

1. Administrative & Study Identification

Full Study Title: A Study of Brentuximab Vedotin and CHP in Frontline Treatment of PTCL With Less Than 10% CD30 Expression
Short Title/Acronym: SGN35-032 **Protocol Number:** SGN35-032 / NCI-2020-09904 **NCT Number:** NCT04569032 **Posting ID:** 929279054357569099 **Sponsor Name:** Pfizer **Investigational Product:** Brentuximab Vedotin (BV) + CHP (cyclophosphamide, doxorubicin, and prednisone) **Phase of Development:** Phase II **Medical Monitor:** Clinical Operations Lead / Medical Monitor, Pfizer

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the objective response rate (ORR) of brentuximab vedotin in combination with CHP (A+CHP) as frontline treatment for patients with peripheral T-cell lymphoma (PTCL) with less than 10% CD30 expression.
Secondary Objectives: To evaluate safety, complete response (CR) rate, progression-free survival (PFS), overall survival (OS), and duration of response (DOR).

2.2 Background & Rationale To address the need for effective frontline therapies in patients with non-sALCL PTCL with <10% CD30 expression. The ECHELON-2 phase 3 trial demonstrated a progression-free and overall survival benefit for A+CHP in PTCL patients with $\geq 10\%$ CD30 expression. Because no correlation was found between the level of CD30 expression and the duration of response, it was hypothesized that frontline A+CHP might also be active and provide meaningful clinical benefit to patients with non-sALCL PTCL exhibiting <10% (or undetectable) CD30 expression.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Control Type:** Single-arm (stratified into two cohorts: CD30 <1% and CD30 1% to <10%) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N\$: 82 patients **Number of Sites:** Multicenter **Estimated Study Start:** 12-Nov-2020 **Estimated Primary Completion:** 09-May-2024

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Newly diagnosed peripheral T-cell lymphoma (PTCL), non-sALCL subtypes, per WHO 2016 classification. Eligible subtypes include: PTCL-NOS, Angioimmunoblastic T-cell lymphoma (AITL), Adult T-cell leukemia/lymphoma (ATLL; acute and lymphoma types only, HTLV-1 positive), Enteropathy-associated T-cell lymphoma (EATL), Hepatosplenic T-cell lymphoma, Monomorphic epitheliotropic intestinal T-cell lymphoma (MEITCL), Indolent T-cell lymphoproliferative disorder (T-LPD) of the GI tract, Follicular T-cell lymphoma, and Nodal PTCL with TFH phenotype. 3. CD30 expression $<10\%$ by local assessment in tumor-containing lymph node or other extranodal soft tissue biopsy. 4. Fluorodeoxyglucose (FDG)-avid disease by PET and measurable disease of at least 1.5 cm by CT. 5. Eastern Cooperative Oncology Group (ECOG) performance status ≤ 2 .

4.2 Exclusion Criteria 1. Current diagnosis of systemic anaplastic large cell lymphoma (sALCL), primary cutaneous T-cell lymphoproliferative disorders and lymphomas, or mycosis fungoides (MF), including transformed MF. 2. History of another primary invasive cancer, hematologic malignancy, or myelodysplastic syndrome that has not been in remission for at least 3 years (exceptions applied for malignancies with negligible risk of metastasis/death, such as carcinoma in situ of the cervix, non-melanoma skin carcinoma, localized prostate cancer, ductal carcinoma in situ, or Stage I uterine cancer). 3. History of progressive multifocal leukoencephalopathy (PML). 4. Cerebral/meningeal disease related to the underlying malignancy. 5. Prior treatment with brentuximab vedotin or doxorubicin. 6. Baseline peripheral neuropathy Grade 2 or higher, or the demyelinating form of Charcot-Marie-Tooth syndrome. 7. Left ventricular ejection fraction $<45\%$ or symptomatic cardiac disease, myocardial infarction within the past 6 months, or previous treatment with complete cumulative dose of $>300 \text{ mg/m}^2$ of doxorubicin. 8. Uncontrolled Grade 3 or higher viral, bacterial, or fungal infection within 2 weeks prior to the first dose of study drug.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: 21-day cycles of A+CHP. Brentuximab vedotin 1.8 mg/kg , cyclophosphamide (ATC Code: L01AA01; Trade names: Cytosan, Endoxana) 750 mg/m^2 , and doxorubicin 50 mg/m^2 administered by IV infusion on Day 1 of each cycle. Prednisone 100 mg orally daily on Days 1-5. **Route of Administration:** Intravenous (IV) for BV, Cyclophosphamide, and Doxorubicin; Oral (PO) for Prednisone. **Duration of Treatment:** Up to 6-8 cycles.

5.2 Concomitant & Prohibited Therapy Permitted: Routine antimicrobial prophylaxis and routine supportive care (e.g., G-CSF for neutropenia management). **Prohibited:** Concurrent

investigational anticancer therapies, prior doxorubicin exceeding a complete cumulative dose of >300 mg/m².

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline LVEF, Local CD30 testing
Treatment (Cycles 1-8)	Day 1 (every 21 days)	IV Infusion of BV+CHP, AE monitoring, Efficacy Assessments (CT/PET)
End of Treatment	End of last cycle	Final Efficacy Assessment, Safety Evaluation
Follow-Up	Post-treatment	Survival Follow-Up (OS/PFS Tracking)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Objective Response Rate (ORR) following the completion of study treatment. **Measurement Method:** Blinded Independent Central Review (BICR) per Revised Response Criteria for Malignant Lymphoma Criteria (Cheson 2007) by Central CD30 Assessment.

7.2 Secondary & Exploratory Endpoints

- Complete Response (CR) Rate per BICR (Cheson 2007) by Central CD30 Assessment
- Progression-Free Survival (PFS) per BICR (Cheson 2007) by Central CD30 Assessment
- Overall Survival (OS) by Central CD30 Assessment
- Duration of Response (DOR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard continuous collection. Particular attention to Treatment-Emergent Adverse Events (TEAEs) such as neutropenia, febrile neutropenia, and peripheral neuropathy. **Serious Adverse Events (SAEs):** Evaluated continuously; Treatment-Emergent Serious Adverse Events (TESAEs) graded per NCI CTCAE Version 4.03. **Data Monitoring Committee (DMC):** Safety profile monitored closely by the sponsor's clinical safety team.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All patients who received ≥ 1 dose of study drug were included in the safety and efficacy evaluations. **Sample Size Justification:** Approximately 82 patients provided adequate power to estimate the efficacy profile (ORR/CR) for these specific subgroups. **Handling of Missing Data:** Time-to-event efficacy endpoints (PFS, OS, DOR) were analyzed using the Kaplan-Meier method.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring by Pfizer.

Audit Trail: eCRF locking and query resolution via standard clinical data management systems.

11. Study Identifiers & Governance

Primary ID: SGN35-032 **Registry Number:** NCT04569032 **Posting ID:** 929279054357569099 **Sponsor/Lead Organization:** Pfizer **Study Title:** A Study of Brentuximab Vedotin and CHP in Frontline Treatment of PTCL With Less Than 10% CD30 Expression **Official Regulatory Title:** A Dual-cohort, Open-label, Phase 2 Study of Brentuximab Vedotin and CHP (A+CHP) in the Frontline Treatment of Subjects With Peripheral T-cell Lymphoma (PTCL) With Less Than 10% CD30 Expression

12. Clinical Framework (PTCL Specific)

Primary Condition: Peripheral T-Cell Lymphoma (PTCL) **Preferred UMLS Name:** CD30-negative Peripheral T-cell Lymphoma **Diagnosis Threshold:** CD30 Expression < 10% **Medical Methodology:** Frontline combination chemo-immunotherapy (A+CHP) **Optimization Target:** Objective Response Rate (ORR) and Complete Response (CR)

13. Study Procedures & Methodology

Management Pathway: Frontline induction therapy (6-8 cycles)

Education Protocol (HCP): Management of adverse events, notably BV-associated peripheral neuropathy and Grade 3/4 neutropenia

Education Protocol (Patient): Infection prevention, chemotherapy side-effect management **Quality Audit Mechanism:** Blinded

Independent Central Review (BICR) of imaging scans

14. Observation & Follow-Up Schedule

Total Duration: Follow-up ongoing until disease progression, death, or study closure (Study Completion estimated Jan 2026)

Onsite Visit Cadence: Day 1 of every 21-day cycle (Cycles 1-8)

Remote Monitoring Frequency: Routine intervals during the post-treatment follow-up phase **Checklist Review Interval:** Every 21 days (per cycle) to evaluate NCI CTCAE toxicities

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					